



Havana Specialist Hospital, Board Operating Model

Prepared for: The Board of Havana Specialist Hospital Limited

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Companion to the "Setting Up the HSH Board" deck. This document turns the open questions in that deck into a working operating model: composition and gaps, the committee architecture, chairmanship, the HSH Board and Afya-HSH Governance Committee relationship, reporting pathways, and compensation. It is written to be handed to the Company Secretary to administer.

1. What the board is for, and what this document does

The deck sets the board four jobs: oversee Afya partnership delivery; secure the long-term growth and sustainability of HSH with or without Afya; build organisational credibility, networks, and institutional influence; and, above all, protect the families' interests.

Those four are the test every governance choice below is measured against. Two points on the fourth. First, HSH is owned across **two founding families**, so "protect the families' interests" means protecting *both*, and balance between them is itself a governance objective: no single family should hold both the executive leadership and the chair, or control the process by which the board renews itself. Second, protecting the families' interests is not in tension with professional governance, it depends on it. The families' shared interest is the long-term value and reputation of the asset. A disciplined, credible board with independent challenge is what protects that value, keeps the Afya relationship honest, and preserves family control where it belongs, through ownership and reserved matters, rather than through day-to-day management.

This document sets out one coherent model so that:

1. the **board** does oversight, strategy, risk, and assurance, and stops short of management;
2. **management and the Afya team** run the hospital day to day within clear delegated authority; and
3. the **committees** in between are sorted into the ones that belong to the board and the ones that belong to operations, so information flows cleanly upward.

2. Principles we are designing to

1. **The board oversees; management operates.** The board sets direction, approves strategy, budget and risk appetite, monitors delivery, and holds management and Afya to account. It does not run the hospital.
2. **Independence provides challenge.** Assurance committees (audit and risk, quality, remuneration and nominations) are led by independent non-executive directors, of whom the board has four.



3. **HSH keeps an independent read on Afya.** The Afya-HSH Governance Committee is jointly run with Afya, so it cannot be HSH's only assurance over Afya's own delivery. The board and its committees retain an independent line of sight. This is the single most important structural point in the whole model.
4. **Report by exception, against a standard.** The board sees a consolidated pack on a fixed template, with commentary on variance and defined escalation thresholds, not raw operational data.
5. **Every committee reports to exactly one parent,** and conflicts and related-party matters (family directors, Afya, paid advisers) are declared, registered, and managed in the open.

3. Composition and the skills gaps

3.1 The board, as it now stands (eight directors, 4:4)

Director	Type	What they bring
Dr Ijeoma Iwuala	Executive, Medical Director	Clinical governance, quality and safety, medical operations (O&G; NHS and Nigeria)
Nwando Okeke	Executive	Legal, regulatory, governance, contracts, health management
Ugo Nwokoro	Executive	Product, technology, digital transformation, business strategy
Okeke Chukwuemeka	Executive	Hospital administration, operations, budgeting, procurement, continuity
Chief Charles Odunukwe	Independent Non-Executive	Corporate leadership, hospitality and investment, financial discipline, networks and community standing
Prof Oliver Ezechi	Independent Non-Executive	Clinical governance, research, academic credibility, quality and clinical systems (infectious disease)
Dennis Olisa	Independent Non-Executive (Board Chair, interim)	Banking, risk, corporate governance; audit, finance and investment oversight (ex-ED, Zenith Bank)
Dr Debo Odulana	Independent Non-Executive	Healthcare systems, operational transformation, organisational structure and innovation strategy

A **4:4 executive-to-non-executive balance** is a strong starting point, and **all four non-executives are independent**. That is unusual, in the good sense, for a family hospital at this stage: it means the assurance committees can be genuinely independent-led, and it gives both families a credible, unaligned counterweight at board level.

3.2 Functional coverage: strong

- Clinical governance and quality: **covered** (Iwuala, Ezechi)
- Finance, audit, risk, investment: **covered** (Olisa, with Odunukwe on investment)
- Legal, regulatory, compliance: **covered** (Nwando Okeke)
- Technology and digital: **covered** (Nwokoro)



- Operations and administration: **covered** (Chukwuemeka)
- Strategy, transformation, organisational design: **covered** (Odulana)
- Credibility, networks, institutional influence: **covered** (Odunukwe, Ezechi)

On the functions a hospital board must hold, the composition is genuinely good. Dennis Olisa brings the finance, audit, risk, and corporate-governance depth the family directors do not themselves hold, and that stature, external and belonging to neither family, is also why he is the chairmanship pick (see section 4). As Board Chair he does not chair a committee; the Audit and Risk Committee is chaired by Chief Odunukwe. So the board's finance and audit expertise and its independent assurance are both covered, without concentrating them in one seat.

3.3 The two gaps that remain

1. **Payer and managed-care expertise, lost with Dr Alli.** Dr Tokunbo Alli (CEO, Leadway Health) was the health-insurance and payer lens, and she withdrew on a conflict of interest. **Dennis Olisa does not backfill this**, his strength is banking and finance, not payer economics. For a Nigerian specialist hospital, HMO contracting, claims, and revenue-cycle economics are central to viability, so this is a real hole, not a nice-to-have. Dr Segun Awoyinfa (managed care, health insurance, health financing) is the profile that would close it.
2. **Nursing, workforce, and accreditation.** No board member carries senior nursing leadership or accreditation and quality-regulation depth. In a hospital, nursing is the largest clinical workforce and the front line of safety. Dr Victoria Hanson is the profile that would close this.

3.4 Recommended position

The board is well composed and Dennis Olisa should be confirmed. Before treating the roster as closed, decide deliberately on the payer gap: either add one further non-executive with managed-care and health-financing depth (Dr Awoyinfa is the strongest fit and also brings transformation), or accept the gap at board level and cover it through management (a strong revenue-cycle and HMO function) plus an expert adviser to the Finance committee. My recommendation is to add Dr Awoyinfa if the board is willing to go to nine, because payer economics sit at the centre of all four board mandates; and to keep Dr Hanson in view for the nursing and accreditation dimension as the hospital scales. Adding both takes the board to ten, which is workable but should be a conscious choice, not a drift.

A note on discipline: eight is a good working size. Every addition past eight should buy a distinct, needed competency, not a duplicate.

4. Chairmanship

The deck asks for a chairmanship recommendation, and it is the most sensitive decision on the table. Four principles, then the recommendation.

1. **The Chair must be separate from the Medical Director.** Dr Ijeoma Iwuala continues as MD and leads the executive. Separating the Chair from the executive is universal good practice: the board cannot hold management to account if the person running the meeting is also the person being held to account.
2. **In a two-family business, the Chair and the top executive should not sit in the same family.** With the MD role held in one family, an independent, non-family Chair is the cleanest way to keep the top of the house balanced, and it protects both families' interests rather than either family's advantage.



3. **The Chair must command three constituencies:** both families, the independents, and Afya. That calls for external stature, financial and governance literacy, and visible neutrality above any single technical skill.
4. **A non-family, independent Chair is therefore the right answer, and Dennis Olisa fits it.** Ex-Executive Director of a major bank, with banking, risk, and corporate-governance depth, he brings the external credibility to steady a new board and to sit across the Afya relationship, and, crucially, he belongs to neither family. Framing his chairmanship as **interim, "Chair for the establishment phase," subject to review at, say, the first annual board evaluation**, keeps it low-pressure and reversible, which matches where the board actually is.

Legitimacy is the real work, not the name. Because the non-executive directors were introduced through one family's initiative, a wary family branch may read even a strong external Chair as that family's proxy. The answer is procedural, not personal: the Chair should be **confirmed by a visible, joint process both families endorse**, not presented as a *fait accompli* (see section 4a). Do that, and Dennis Olisa is an asset both families can own.

Dr Odulana should not chair the board. Consult for Africa is a paid adviser to HSH on this very work, so chairing would concentrate too much and blur the adviser and director roles. The clean placement is independent director, member of the assurance committees, and recused from any decision on Consult for Africa's own fees or scope. This same transparency is what lets Dr Odulana act as an honest broker on composition and process.

Recommendation: Dennis Olisa as (interim) Non-Executive Chair, confirmed through a joint, both-families process, with Prof Ezechi as Lead Independent Director as a balancing presence at the top table. (King IV recommends a Lead Independent Director as a matter of course, precisely to give the other directors a stronger voice where the chair is dominant or not independent, and to mediate conflict involving the chair.)

4a. Making the appointment legitimate to both families

The path of least resistance is not to argue about the person, it is to make the *process* unimpeachable:

- **Let the non-partisan directors carry it.** Chief Odunukwe and Prof Ezechi are familiar with and trusted by both families, and read as neutral. They, not the directors associated with getting the board started, are the right voices to propose and vouch for the Chair and the process. When the two people both branches trust endorse the appointment, it is legitimised without anyone having to argue it.
- **Both families formally endorse the Chair.** The appointment is proposed through the People and Remuneration / Nominations committee and ratified by the board, with each family's directors on record supporting it. A chair installed by consensus cannot later be dismissed as one side's man.
- **Balance the top table.** Pair the Chair with a Lead Independent Director, and give the People and Remuneration committee (which controls future appointments and fees) to the family that does not hold the MD. This is where a wary branch most needs a real seat, and it is why the RemCo chair goes to Nwando Okeke.
- **Reserved matters and a fair-process rule.** Codify that major decisions (section 8.4) need genuine deliberation and, where a family has a material interest, that the Chair actively seeks both families' voice before a decision. Fair process, being heard and knowing why a decision was made, buys more peace than any single appointment.
- **Re-own the board's composition.** The board was convened through the initiative of directors who moved to end a period of drift, which is to their credit. The next step turns that initiative into shared ownership: the full board formally adopts its own membership and terms of reference through the



Nominations process, so it becomes plainly *everyone's* board rather than anyone's project.

5. The committee architecture: two tiers, one rule

The deck already sketches board-level committees and, separately, the operational structure under the Afya-HSH Governance Committee (Performance Review, Quality Assurance, Hospital Manager, Clinical Administrator). The reconciliation is to name the rule that separates them:

A committee is a board committee only if the board has delegated it authority to oversee or assure on the board's behalf, and it reports to the board. Everything else is an operating committee: chaired by an executive or by Afya, meeting on an operational cadence, reporting into the Afya-HSH Governance Committee, and reaching the board only as a consolidated report.

So there are two tiers:

- **Tier 1, board committees** (few: five recommended), non-executive led, reporting to the board.
- **Tier 2, operating committees** (as many as operations needs), the structures Afya sets up beneath the Governance Committee, reporting into it.

When Afya's committee proposal lands, sort each item with this rule. The expectation, and it matches the instinct in your email, is that most of what Afya proposes is Tier 2 and should not report directly to the board. The operational functions already in the deck, Quality Assurance, Hospital Manager, Clinical Administrator, and the Performance Review Subcommittee, are all Tier 2.

6. Tier 1: the five board committees

Recommended set, aligned to the deck's own naming, with placements and cadence. Composition is a proposal for the board to confirm.

6.1 Finance, Investment and Operations Committee (FIOC)

- **Purpose:** oversight of financial performance, capital and investment decisions, and operational delivery. This is the board's performance committee, distinct from the assurance provided by Audit and Risk.
- **Co-chairs:** Okeke Chukwuemeka (Family B; operations, budgeting, administration) and Ugo Nwokoro (Family A; strategy, technology). The money committee is deliberately co-chaired by one director from each founding family, so financial and operational oversight is shared and neither family can dominate it. The Board Chair does not chair, so he is not double-hatting and presides over the board impartially.
Members: Chief Odunukwe (investment), Dr Odulana or a further NED. Hospital Manager / Head of Finance, Operations, and the Afya finance lead attend by invitation.
- *How the co-chairs work: they agree the order of business and alternate or co-preside; a decision they cannot agree on is referred to the full Board rather than settled by a casting vote. Both sit as directors providing oversight of performance delivered by Afya and management, not in any executive line capacity, so it is not self-review. The Board Chair (Dennis Olisa) may attend but does not chair.*
- **Cadence:** quarterly.
- **Remit:** management accounts and financial performance; budget monitoring; capital and investment appraisal; operational KPIs and delivery; the financial and commercial performance of the Afya arrangement (with an independent read retained by Audit and Risk).



6.2 Audit and Risk Committee (ARC)

- **Purpose:** independent assurance over financial reporting, internal controls, internal and external audit, the enterprise risk register, and compliance. The committee that holds the finance function and management to account, so it sits apart from them.
- **Chair:** Chief Charles Odunukwe (financial discipline, independent, trusted by both families). Deliberately **not** the Board Chair and **not** the chair of the Finance committee, so assurance is independent of both. **Members:** Nwando Okeke (legal, compliance, related-party scrutiny), Dr Odulana (recused from Consult for Africa related-party items). Head of Finance and internal/external audit attend by invitation.
- **Cadence:** quarterly, plus an audit-planning meeting. Five a year.
- **Remit:** annual financial statements and going-concern judgement; internal controls and the internal audit plan; appointment and evaluation of the external auditor; the enterprise and financial risk register; related-party transactions (family and Afya); the independent read on Afya's financial delivery.
- *Governance note (IFC / King IV): the audit committee is the board's primary independent-assurance mechanism. Chairing it with a neutral, non-family, finance-literate director is what lets both families trust the numbers.*

6.3 Quality and Clinical Governance Committee (QCGC)

- **Purpose:** assurance that the hospital delivers safe, high-quality clinical care and that clinical risk is managed.
- **Chair:** Prof Ezechi (independent, clinical credibility; also Lead Independent Director). **Members:** Dr Iwuala (Medical Director, as executive clinical lead reporting in). Dr Hanson if appointed (nursing, accreditation).
- **Cadence:** quarterly.
- **Remit:** clinical quality and patient-safety metrics; incident and near-miss reporting and learning; consolidated review of the Tier 2 clinical functions (Clinical Administrator, Quality Assurance); credentialing and protocols; clinical risk (feeding the single enterprise risk register owned by Audit and Risk); regulatory and accreditation readiness.
- *On risk: one enterprise risk register, owned by Audit and Risk, read through two lenses, financial and control risk at Audit and Risk, clinical and operational risk here.*

6.4 People and Remuneration Committee (RemCo / Nominations)

- **Purpose:** board composition and succession, appointments, executive and director remuneration, board evaluation, workforce strategy, and conflicts management.
- **Chair:** Nwando Okeke (legal, governance, NBA). Chairing this committee is deliberately given to the family that does not hold the Medical Director role, so the committee controlling appointments and remuneration is a visible counterweight rather than another lever in one family's hands. **Members:** Prof Ezechi (Lead Independent Director), one executive director from the other family, Dr Odulana (recused from Consult for Africa fee matters).
- **Conflict guardrail:** because the chair is an executive director, a hard recusal rule applies. The chair and any director step out of decisions on their own and close family's remuneration, and the independent non-executives carry those items. Nominations (composition and appointments) may be led by the chair; sensitive remuneration decisions are taken by the independents present.
- **Cadence:** twice a year, plus as needed for appointments.
- **Remit:** the skills-gap and appointment pipeline (this is where the Awoyinfa and Hanson decisions are formally handled); induction and board evaluation; executive and senior remuneration; the **board**



sitting-allowance and participation policy (section 9); the conflicts and related-party register.

6.5 Transformation and Digital Committee (time-boxed)

- **Purpose:** oversight of the digital and operational transformation during the current build, in support of the growth and sustainability mandate.
- **Chair:** Dr Debo Odulana (independent non-executive). A non-executive chairs, so the committee provides oversight rather than being run by the executive delivering the work. **Members:** the relevant executives, Head of Operations, and the Afya technology lead in attendance; external adviser as needed.
- **Conflict guardrail:** Dr Odulana recuses from any decision where Consult for Africa or CadreHealth could deliver or bid for the work, on the same transparency standard as his fee recusal.
- **Cadence:** monthly while active, then quarterly.
- **Remit:** technology and digital roadmap, major system selection and implementation, data governance and information security, digital patient experience.
- **Design note:** constitute this with a **defined review or sunset date**. It is the committee most likely to migrate to Tier 2 (operations) once transformation stabilises, so it should not become permanent by default. A worked example for Afya of how a real, important function can still be time-boxed rather than a standing board committee.

7. The HSH Board and the Afya-HSH Governance Committee

The deck's two-layer model is sound, and this sharpens it.

7.1 The two layers

- **Layer 1, HSH Board.** Strategic oversight. Meets quarterly. Holds final approval of budgets, major investments, and risk appetite. Owns the reserved matters. Ultimate accountability for the hospital and for the families' interests.
- **Layer 2, Afya-HSH Governance Committee.** Operational governance of the partnership, jointly staffed by the Afya Care and HSH executives. Meets monthly. Tracks execution, KPIs, and turnaround progress through its Performance Review Subcommittee and the operating functions (Quality Assurance, Hospital Manager, Clinical Administrator). Escalates to the board.

7.2 The relationship, drawn cleanly

- The Governance Committee has **delegated operational authority within limits set by the board** (the reserved matters, section 8). It decides within those limits and no further.
- It **reports upward to the board on a fixed template**, and its financial and clinical performance is also read independently by the Audit and Risk and the Quality and Clinical Governance committees.
- **Reserved matters stay with the board.** Budget approval, major capital, changes to the Afya agreement, senior clinical and executive appointments, opening or closing service lines, and borrowing cannot be settled in the joint committee alone.

7.3 The point that must not be missed

Because the Governance Committee is **jointly controlled with Afya**, it cannot be HSH's only assurance over Afya's own delivery. If the board only ever sees Afya's performance through a body Afya co-chairs, oversight of mandate one (Afya delivery) is compromised. So:

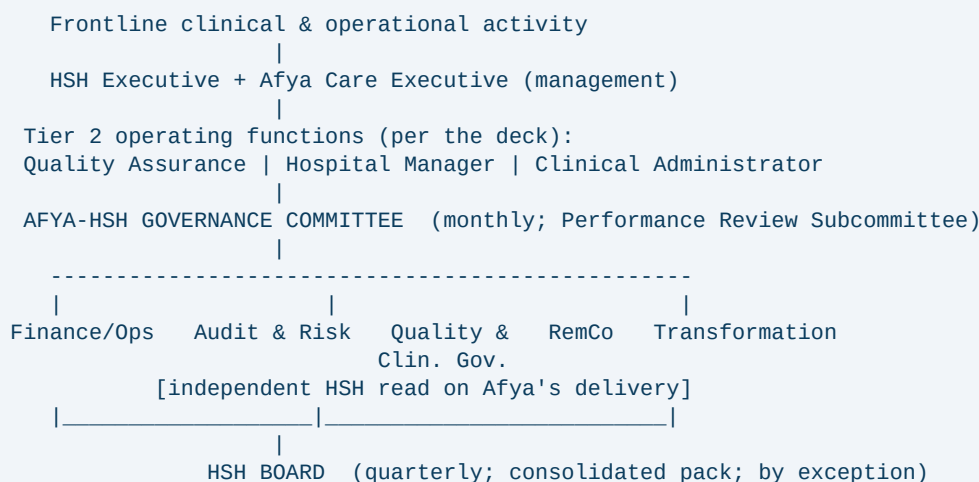


- The board's own committees (Audit and Risk on the numbers, Quality and Clinical Governance on clinical quality) provide an **independent HSH read** on Afya's performance.
- The full board holds a **standing quarterly "Afya partnership performance" item**, informed by both the Governance Committee's report and its own committees.
- The families' interest in the "with or without Afya" mandate is protected by keeping this independent line of sight live from day one.

8. Reporting pathways and reserved matters

8.1 The upward flow

Frontline and operations → Management (HSH exec + Afya) → Tier 2 operating functions → Afya-HSH Governance Committee (monthly) → board committees + the board (quarterly).
Information is consolidated and filtered at each step, so it reaches the board decision-ready.



8.2 How information is filtered

1. **Standard templates.** Each layer reports upward on a fixed one-page template: metrics against target, exceptions, risks, decisions needed. No free-form dumps.
2. **Exception-based escalation with pre-agreed thresholds.** Serious clinical incidents escalate immediately; budget variance beyond a set percentage escalates to Finance, Investment and Operations; risks above a set rating escalate to Audit and Risk and to the board.
3. **One consolidated board pack,** assembled by the Company Secretary: Chair's agenda, management report, Governance Committee report, each committee chair's report, risk-register summary, finance pack. Circulated seven days ahead.

8.3 Cadence

- **Board:** quarterly (as the deck proposes), plus one annual strategy session. During the transition, consider six-weekly for the first two or three meetings, then settle to quarterly.
- **Afya-HSH Governance Committee:** monthly.
- **Committees:** Finance, Investment and Operations quarterly; Audit and Risk quarterly plus audit planning; Quality and Clinical Governance quarterly; RemCo twice a year; Transformation monthly then quarterly.



- The Company Secretary publishes a rolling twelve-month calendar so committees feed the board in the right order.

8.4 Reserved matters and delegation

A one-page Delegation of Authority schedule, owned by RemCo and administered by the Company Secretary, specifies what the board keeps and what it delegates:

- **Reserved to the board:** strategy and annual budget; capital above a threshold; any change to the Afya partnership or management agreement; senior executive and Medical Director appointments; opening or closing service lines; borrowing; related-party transactions above a threshold; distributions.
- **Delegated to management and the Governance Committee:** operational spend within budget and thresholds; day-to-day clinical and operational decisions; hiring below senior level; suppliers within policy.

This schedule is what protects the families' interests in practice: it draws the line the Afya-HSH Governance Committee cannot cross, and keeps the strategic levers with the board and the owners.

9. Board compensation

The deck proposes a sitting allowance of N250,000 per meeting, to rise as performance improves.

- **Board-only basis:** $N250,000 \times 4 \text{ board meetings} = \text{N1,000,000 per director per year}$.
- **With committee attendance:** $N250,000 \times (4 \text{ board} + \sim 6 \text{ committee attendances}) \approx \text{N2,500,000 per director per year}$.
- Governance Committee meetings (twelve a year) are informational for directors who are not standing members, and should not attract a sitting allowance unless the director is an appointed member.

Recommendation: adopt the participation-based model, but have **RemCo set a clear participation and payment policy**, which meetings attract an allowance, expected attendance, and the performance conditions for future increases, so compensation tracks contribution rather than attendance alone. Keep NED fees modest and transparent while the turnaround is underway; revisit annually.

10. Next steps

1. **At today's meeting:** agree the two-tier principle, the five board committees and their chairs, the chairmanship recommendation (Dennis Olisa as interim Chair, Ezechi as Lead Independent Director), the quarterly board cadence, and confirmation of Dennis Olisa.
2. **Decide the payer gap:** whether to add Dr Awoyinfa (recommended) and whether to keep Dr Hanson in view.
3. **Reconcile with Afya:** sort their committee proposal into Tier 1 and Tier 2 against the rule in section 5.
4. **Draft the instruments:** committee terms of reference, the reserved-matters and delegation schedule, reporting templates, the board calendar, and the compensation and participation policy.
5. **Hand to the Company Secretary** to administer, and set the date to kick off board operations.

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